



Words in the White Coat

Grade 8 • North Carolina • Reading

RI.8.4

Determine figurative, connotative, and technical meanings of words and phrases in an informational text, and analyze how specific word choices shape meaning and tone. An 80-minute block-period packet with warm-up, guided practice, partner discussion, independent practice, small-group work, and a full answer key.

PRINT • TEACH • REVISIT



Plan the 80-minute block

Level and alignment

Grade 8 · North Carolina · Reading; RI.8.4 — Determine the meaning of words and phrases as they are used in a text, including figurative, connotative, and technical meanings; analyze the impact of specific word choices on meaning and tone, including analogies or allusions to other texts.

What is included

Cover; teacher guide; vocabulary and lesson preview; original passage; warm-up (2 items); guided practice with organizer (4 items); partner discussion (3 items); independent practice (6 items); small-group intervention and enrichment (4 items); exit ticket; complete answer key; access and sources page.

Suggested pacing (80 minutes)

Warm-up 8 min · passage + guided practice 25 min · partner discussion 12 min · independent practice 20 min · small-group work 10 min · exit ticket 5 min.

Preparation

Print student pages separately from the answer key. Review the passage and organizer before class. No outside novel or paid app required.

Student directions

Read the passage carefully. Use evidence from the text to support every answer. Quote or closely paraphrase the text and name the paragraph.



Before you read

Key vocabulary

Lesson focus

Informational texts often mix technical meanings (precise, field-specific definitions) with connotative meanings (the feelings a word carries). At grade 8, notice when a writer chooses a more clinical, technical word versus a more everyday or emotionally loaded one, and consider what effect that choice has on the reader.

Model 1 — A medical report could describe a patient as "obese" or as "carrying extra weight." What is the difference between these two word choices?

"Obese" is a precise technical/clinical term with a specific medical definition, while "carrying extra weight" is a softer, more everyday phrase; the technical term sounds more clinical and less personal, while the everyday phrase may feel gentler but less precise.

Model 2 — If a doctor says a treatment has a "statistically significant effect," what technical meaning does "significant" have here that differs from its everyday connotation of "important" or "large"?

In statistics, "significant" technically means the result is unlikely to be due to random chance, which is a specific mathematical claim; it does not necessarily mean the effect is large or clinically important, which is how the word is often used in everyday speech.



WARM-UP

Warm-up · Before you read

Answer using what you already know. Five minutes.

1. What explicit word does paragraph 1 identify as Dr. Morrow's first choice when a diagnosis is still uncertain?

2. What technical reason does paragraph 1 give for choosing "mass" instead of more specific terms?



Words in the White Coat: How Doctors Choose Language for Patients (Part 1)

1. When Dr. Elena Morrow was a medical resident, a senior physician told her something she still repeats to her own students two decades later: "You have exactly one chance to choose the first word a patient hears about their diagnosis, and that word will color everything they hear after it." The word, in that case, was "mass." Not "tumor," not "growth," not "cancer" — just "mass," a technically accurate but deliberately neutral term used until further testing could clarify what kind of mass it was.

2. Medical language operates on at least two levels at once: the technical level, where a word has a precise, field-specific definition regardless of how it sounds, and the connotative level, where the same word carries emotional weight a patient will feel immediately. "Tumor" is technically neutral — it simply means an abnormal mass of tissue, which can be benign or malignant — but its everyday connotation is almost entirely frightening, often assumed to mean cancer even when it does not.

3. This gap between technical meaning and connotation creates real dilemmas. A 2021 survey of oncologists found that 68 percent said they deliberately chose less alarming words during an initial diagnosis conversation, even when a more clinical term was technically more precise, specifically to avoid overwhelming a patient before they were ready to absorb full information.



Words in the White Coat: How Doctors Choose Language for Patients (Part 2)

4. Word choice matters even in written reports patients read themselves. Dr. Morrow points to the shift many hospitals have made from "obese" to "BMI above 30" or "elevated BMI" in visit summaries. The number is more technical and, according to Morrow, "somehow feels less like a judgment and more like a measurement," even though both phrases describe the identical clinical finding.

5. Some word choices rely on analogy rather than technical precision. Dr. Morrow often describes chemotherapy to patients as "clearing a garden that has both the plant you want and the weeds choking it out — the tools cannot always tell the difference." She avoids the still-common military analogy of "fighting" or "battling" cancer, which she says can unintentionally suggest a patient who dies from the disease somehow "lost," as if survival were purely a matter of effort.

6. Technical terms are not always more comforting than emotional ones. When Dr. Morrow tells a patient a treatment showed a "statistically significant" improvement, some patients hear "significant" and assume a dramatic, life-changing result, when the technical meaning only confirms the result was unlikely to be due to chance and says nothing about its actual size.

7. Morrow's current guidance to her residents is not to avoid technical language altogether, but to name the gap out loud: to say, in effect, "here is the precise medical term, and here is what it does and does not mean for you," treating word choice itself as part of patient care rather than a formality that happens before the real conversation begins.



Guided practice · Evidence organizer

Work through this organizer together before independent practice.

It Says	It Means	And So
Quote or closely paraphrase the text.	Explain the evidence in your own words.	State the inference or analysis it supports.

1. Explain the distinction paragraph 2 draws between a word's technical meaning and its connotative meaning, using "tumor" as the example.

2. What explicit statistic does paragraph 3 provide about oncologists' word choices during initial diagnosis conversations?

3. Why, according to paragraph 3, might a doctor choose a less precise word over a more technically accurate one?

4. What explicit shift in terminology does paragraph 4 describe hospitals making, and what technical meaning does the new term have?

Partner discussion

Discuss each question with a partner before writing a joint response.

1. According to Dr. Morrow's quote in paragraph 4, what is the connotative effect of using a number ("BMI above 30") instead of a descriptive word ("obese")?

2. Identify the analogy Dr. Morrow uses to describe chemotherapy in paragraph 5, and explain what two things it compares.

3. Why does Dr. Morrow avoid the "fighting" or "battling" analogy for cancer, according to paragraph 5?



Independent practice

Work independently. Cite evidence from the text in every answer.

1. What is the technical meaning of "statistically significant," as explained in paragraph 6?

2. How does the everyday connotation of "significant" in paragraph 6 differ from its technical statistical meaning, and why does this matter for patients?

3. What is Dr. Morrow's current guidance to her residents, according to paragraph 7?



Independent practice

Work independently. Cite evidence from the text in every answer.

4. Why does the article end with Dr. Morrow's guidance in paragraph 7 rather than with the chemotherapy analogy in paragraph 5? What does this suggest about the article's central point?

5. Compare the tone created by "mass" (paragraph 1) to the tone created by "tumor" (paragraph 2), even though both can technically describe the same thing.

6. A reader claims doctors should always use the most technically precise word possible, regardless of connotation. Using evidence from the article, explain why Dr. Morrow's practice complicates this claim.

Small-group work

Use with students who need additional support.

1. What is the technical, field-specific meaning of "BMI," and how does paragraph 4 suggest the number format changes the tone of a clinical finding compared to a descriptive word?

2. Explain the figurative comparison in paragraph 5 between chemotherapy and gardening, identifying what each part of the analogy represents.



Small-group work

Use with students ready for additional challenge.

1. What connotation does the "fighting" or "battling" analogy for cancer carry, according to Dr. Morrow in paragraph 5, and why does she consider it potentially unfair?

2. Compare the connotative effect of the gardening analogy in paragraph 5 to the connotative effect of the "fighting" analogy she avoids. Why might one feel gentler than the other?



Exit ticket

Answer independently to show what you learned today.

1. Explain how the article's title, "Words in the White Coat," uses connotation to introduce its subject before the reader reaches paragraph 1.



Exit ticket · self-assessment

Rate today's work:

4 — I can teach this to someone else.

3 — I can do this independently.

2 — I can do this with help.

1 — I need to revisit this with my teacher.



Answer key

1. "Mass."
2. It is technically accurate but deliberately neutral, used until further testing can clarify what kind of mass it actually is.
3. Technically, "tumor" simply means an abnormal mass of tissue that can be benign or malignant; connotatively, most patients hear the word and immediately assume cancer, even though that is not what the technical definition requires.
4. 68 percent of oncologists surveyed in 2021 said they deliberately chose less alarming words, even when a more clinical term was technically more precise.
5. To avoid overwhelming a patient with alarming language before the patient is ready to absorb full information about their condition.

Accept equivalent, well-supported alternative answers throughout.



Answer key

6. A shift from "obese" to "BMI above 30" or "elevated BMI," where BMI is a specific, calculated numerical measurement rather than a descriptive label.
7. It "feels less like a judgment and more like a measurement," even though both phrases technically describe the same clinical finding.
8. She compares chemotherapy to "clearing a garden that has both the plant you want and the weeds choking it out," comparing healthy and cancerous cells to desired plants and weeds, and the imprecision of treatment to a gardening tool that cannot always tell them apart.
9. She says it can unintentionally suggest that a patient who dies from the disease somehow "lost," implying survival is purely a matter of effort, which she considers an unfair and inaccurate connotation.
10. It technically means the result is unlikely to be due to random chance; it says nothing about how large or dramatic the actual effect is.

Accept equivalent, well-supported alternative answers throughout.



Answer key

11. Everyday "significant" suggests something large or dramatic, while the technical meaning only confirms the result is not due to chance; this matters because patients may wrongly expect a dramatic improvement based on the everyday connotation alone.
12. Not to avoid technical language, but to explicitly name the gap between the technical term and what it does and does not mean for the patient.
13. Ending with her guidance suggests the article's central point is not just that word choice matters, but that doctors should be deliberate and transparent about it, treating the choice of words as an active, ongoing part of patient care rather than a single clever word or analogy.
14. "Mass" creates a calm, neutral, even slightly vague tone, buying time before a clearer picture emerges, while "tumor" creates an alarmed tone because of its strong connotative association with cancer, even when the underlying finding is identical.
15. Dr. Morrow deliberately avoids the most alarming precise term in initial conversations (paragraph 1) and studies show most oncologists do the same (paragraph 3), showing that in practice, doctors weigh a word's emotional impact alongside its technical precision rather than always choosing the most exact term available.

Accept equivalent, well-supported alternative answers throughout.



Answer key

16. BMI (body mass index) is a specific numerical measurement calculated from height and weight; paragraph 4 suggests presenting it as a number feels more neutral and measurement-like, while a descriptive word like "obese" can feel more like a personal judgment, even though both describe the same finding.

17. The "plant you want" represents healthy cells, the "weeds choking it out" represent cancerous cells, and "the tools cannot always tell the difference" represents chemotherapy's inability to perfectly distinguish healthy from cancerous cells, explaining a complex medical limitation through a familiar, everyday image.

18. It carries a connotation that surviving is a matter of effort or willpower, like winning a fight; she considers it unfair because it implies a patient who dies somehow "lost" or did not try hard enough, when survival often depends on factors beyond a patient's control.

19. The gardening analogy focuses on the imprecision of a tool doing careful work, which feels gentler and more forgiving, while the fighting analogy frames the patient as personally responsible for winning or losing, which can feel more like praise or blame directed at the patient.

20. The "white coat" is a strong visual and cultural symbol of medical authority and expertise, so pairing it with "words" in the title immediately signals that the article is specifically about how that authority is expressed and shaped through language choices, not just medical facts themselves.

Accept equivalent, well-supported alternative answers throughout.

Access, sources, and license

AIG

Ask for a second interpretation, a counterexample, or a student-written item at the same rigor, with an explanation of the choice.

504

Follow the individual plan: extended time, breaks, enlarged print, reduced copying, or an alternate response method as documented.

EC

Pre-teach key vocabulary before reading. Model the first guided-practice item aloud. Accept oral or dictated responses when appropriate.

ESL

Preview key vocabulary with examples. Allow brief home-language discussion before independent writing. Provide sentence starters for open-response items.

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NC ELA Standard Course of Study, Grade 8: <https://www.thecorestandards.org/ELA-Literacy/RI/8/4/>

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